VALSARTAN

Teratogenic

Heart failure benefit

CHEMICAL NAME (2S)-3-methyl-2-[pentanoyl({4-[2-(1H-tetrazol-5-yl)phenyl]phenyl}methyl)amino]butanoic acid

CLASS Angiotensin II receptor blocker (ARB)

TRADE NAMES Diovan · Valzaar · Co-Diovan (with HCTZ) · Exforge (with amlodipine)

THERAPEUTIC USES / INDICATIONS

- Hypertension.
- Heart failure (NYHA II–IV) when ACE inhibitor not tolerated.
- Post-myocardial infarction LV dysfunction.

PHARMACODYNAMICS

- Blocks **AT1 receptors** → vasodilation, ↓ aldosterone, ↓ sympathetic activity.
- Reduces cardiac remodelling.

ADMINISTRATION

- **PO** once or twice daily, consistently with or without food.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Dizziness, headache.
- Fatigue, diarrhoea.
- Hypotension.

ADVERSE EFFECTS (SERIOUS)

- Hyperkalaemia.
- Renal impairment.
- Angioedema (rare).
- Neutropenia (rare).

DOSAGES

- **Hypertension:** 80–160 mg once daily; max 320 mg.
- **Heart failure:** 40 mg twice daily, titrate to 160 mg twice daily.
- **Post-MI:** 20 mg twice daily, titrate to 160 mg twice daily.
- **Children 6–18 yr:** 40–80 mg daily by weight.

CONTRAINDICATIONS & CAUTIONS

- Pregnancy.
- Severe hepatic impairment, biliary cirrhosis, cholestasis.
- Aliskiren in diabetes.

KEY INTERACTIONS

- **K^+ supplements, spironolactone:** hyperkalaemia.
- **NSAIDs:** AKI.
- **Lithium:** toxicity.

PHARMACOKINETICS

- **Onset:** 2 h; full effect 2–4 weeks. **Duration:** 24 h.
- Minimal metabolism.
- Excreted mainly in bile/faeces. **t½:** 6–9 h.

NURSING MANAGEMENT

- Monitor BP, K^+, creatinine.
- HF: daily weight, oedema, breathlessness.
- Hold if hypotension.
- Fall precautions in elderly.

PATIENT EDUCATION

- Take regularly at the same time.
- Rise slowly; report fainting.
- Avoid NSAIDs and potassium supplements.
- Avoid pregnancy.

KEY POINTS

- ARB used in hypertension and HF.
- Biliary excretion.
- Component of sacubitril/valsartan.
- Hyperkalaemia; contraindicated in pregnancy.